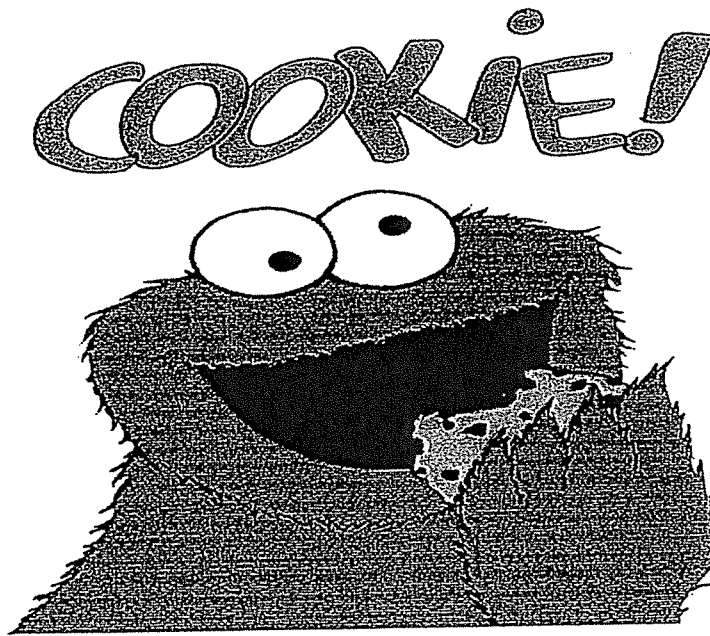




Roseway Training Cookie check for mobile hoists

C – CASTORS (for overhead hoists – Cuts and Frays)

O – OPERATION (Operate all moving parts)

O - OIL AND BOLTS

K - KILOS

I – INSPECTION (Date of service)

E - ENVIRONMENT

GENERAL PRINCIPLES OF SAFER

HANDLING

Being aware of the basic principles will help reduce injury.

Moving and handling is when you help or assist a person to help themselves. Lifting is when the client cannot assist and the carers take the whole body weight. Therefore, your main aim should always be to help clients move themselves.

Never move or lift unless it is necessary (If lifting is necessary, you should use a hoist or other identified equipment).

Always assess what you are going to do before you do it.

Prepare the handling area, make sure you have enough room to move and the area is free of obstacles.

Wear appropriate clothing, which allows free movement and suitable flat footwear.

Check the individual care programme and manual handling risk assessment prior to moving and handling any client.

Explain to the client and any person assisting in the manoeuvre exactly what is going to happen.

Make a good stable base with your feet, which is feet apart approximately shoulder width, with one foot pointing in the direction of the move.

Always keep the weight close to the body.

Always select the appropriate equipment for the task in hand e.g. slide sheet, handling belt, slide board.

When team handling, one person should take the lead. You should decide who before commencement, to avoid any confusion.

The leader should give clear instructions. Ready, Steady and then the manoeuvre; move/sit/stand.

- Maintain a good posture and raise the head on commencing the manoeuvre to ensure your spine is in the correct position, and to ensure you have a clear view.
- Always lift with your legs and NOT YOUR BACK, so bend the knees and keep the back straight when moving and handling.
- Avoid twisting or rotation of the waist, as bending and twisting during handling is one of the most common causes of injury.
- If in any doubt regarding any aspect of moving and handling seek advice.

COMMUNICATION

Before moving and handling you should consider the importance of your communication with the person. The importance of good communication at every stage cannot be emphasised enough. Communication is vital from the initial point of assessing needs to the carrying out of each element of day to day care.

It must be remembered that communication is a two-way process. As a carer, you need to be aware that every person is different. You must fully explain to the person what is going to occur and when. It is also important to listen. Try to put yourself in the other person's shoes, listen to what they are saying so you can address their particular fears or worries. It is important that the person feels that they have some control over what is happening to them. This is why it is so important to communicate clearly and listen.

Good communication is important as part of a carers approach to reducing risk. By explaining clearly what needs to be done, how and why, you can help minimise the unexpected. Communication prepares both person and carer.

It is very important that you are aware that each person who needs care, whatever their illness or disability is likely to feel some sort of anxiety or fear as a result of their changed situation. Such fears may

be temporary, or they may be ongoing or recurring, they may be part of the illness itself. For example, following a stroke, a person may lose the ability to speak but this does not mean they are unable to understand what is going on around them. You need to have a great deal of patience in explaining what is planned to help prevent the frustration or anxiety felt by the person. Communication is also important for providing reassurance and making the person feel safe.

Another example, a person with a hearing impairment may not use their hearing aid due to pain or discomfort, or it may simply not be functioning correctly. It would be pointless approaching, or trying to communicate with the person if you are at their side or back. Always approach the person from the front and face them squarely when communicating with them. In both cases you may have to explain things several times; repeating previous explanations and taking care to be very patient. You must guard against making assumptions based on your own personal views. Try to see the situation from the person's point of view, think how you may feel if it were you in their position. Sometimes fear or lack of understanding due to dementia, about what is happening can lead to some form of anxiety. At all times your approach should be appropriate to the specific needs and circumstances of the person involved. For many people clear, calm and reassuring communication will allay most of their fears.

It is important that you have a positive approach, for example when suggesting the use of a particular piece of equipment, explain what it is for and the benefits to the person. You need to explain, for instance how the use of a belt can enable the person to be assisted to move from armchair to wheelchair in a comfortable and safe manner.

Remember that communication is one of the basic and most important things in the manual handling process and in providing a good standard of care.

SAFE TECHNIQUES

Fig. 8.7

*Assisting to stand
with palm to
palm and
handling belt*

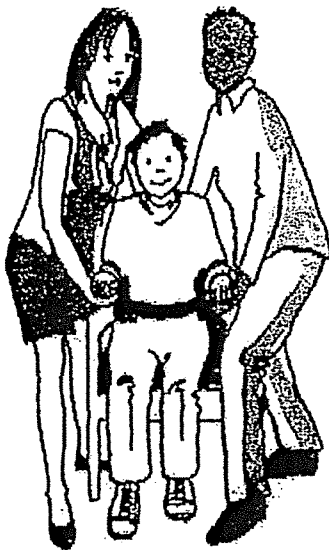


Fig. 8.8

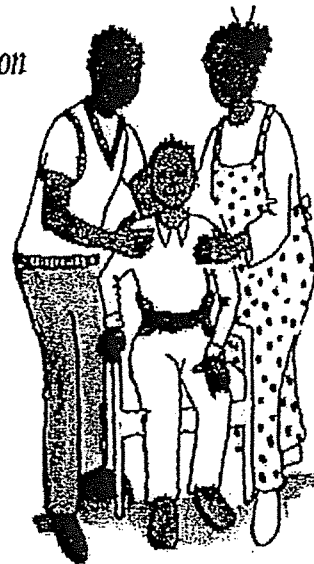
*Assisting to stand
supporting elbow and
using handling belt*



2 carers



*Person - hand on
chair arm*



Standing with assistance from chair.

One or two carers dependent upon the person's abilities and assessment may carry out this manoeuvre.

Confirm person's ability to weight bear, do they pick feet up?

Inform all parties involved on manoeuvre. Confirm all persons understand. Carer positions feet, back foot level with persons hip,

front foot level with persons feet pointing in direction of movement. Carer holds their hand 'Palm to Thumb' ('Palm to Palm'/'Elbow' or Shoulder Support') and places other arm around person's back/waist. Get in as close to the person as possible. Assist forward into the 'nose over toes' position.

Give command, Ready, Steady, Stand. Push forward and down to stand, carer transferring weight from back foot to front.

If only one carer, ask the person to assist by pushing down on carers hand, and /or chair arm. For more stability a handling belt can be placed around person for carer/s to grasp. Once standing give person walking aids or assist to walk.

Walking with assistance

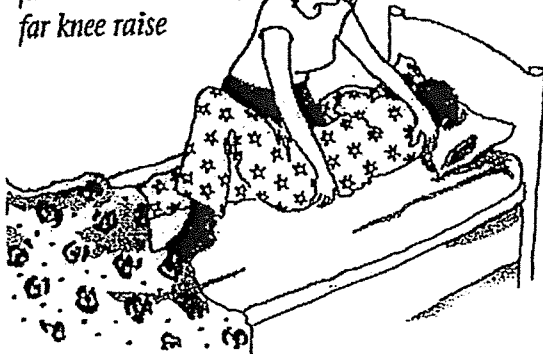
Hold the their hand 'Palm to Thumb' / Palm to Palm' at approximately hip height. Carer places other hand around person's waist. If they require assistance of two carers, they should cross their arms over the person's back. Carer positions themselves as close to the person as possible, good stable base with feet pointing in direction of movement. Always use a handling belt for more stability.

Handling Belts

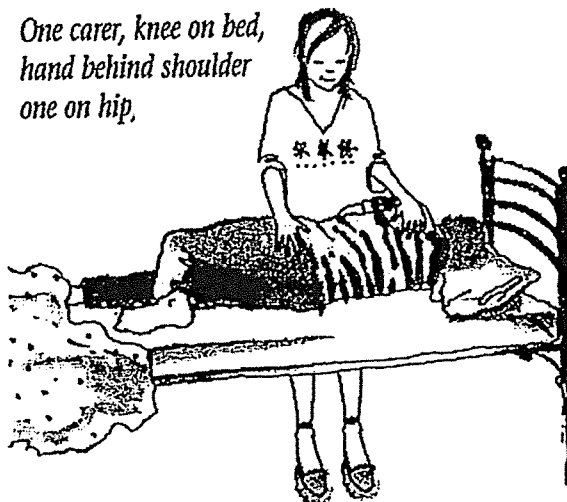
Ensure that when they are placed around a person's waist that they are secure enough to avoid slipping. If a belt is not secure it will ride up the person's body, then you will be drag lifting.

Also ensure that you never grasp a handling belt in a way which could prevent you releasing in case of emergency, e.g. assisting a person to fall.

*Person on side with
near arm up on pillow,
far arm across chest,
far knee raise*



*One carer, knee on bed,
hand behind shoulder
one on hip,*



Rolling in bed with assistance

Note: ALWAYS ROLL PERSON TOWARDS YOU NEVER AWAY UNLESS THERE IS A SECOND PERSON AT OPPOSITE SIDE OF BED.

If possible raise the bed to best working height.

Ensure if fitted, bed brakes are applied.

Inform all parties involved on manoeuvre. Confirm person understands.

Carer places bent knee on edge of bed level with person's waist

Position the person: head turned in direction of move, arms folded across chest, leg furthest away from you flexed up.

Carer places one hand on the persons hip and other on person's shoulder blade.

Give command Ready, Steady, roll. Proceed to roll person.

CARER SHOULD NOT TAKE THEIR KNEE OFF THE BED.

Reposition person.

Alternatively if the person cannot flex their knee through disability or pain, cross their ankles in direction of move. Place one hand on hip and other on shoulder blade and roll.

Lying to sitting on the bed

One or two carers dependent upon the person's abilities and assessment may carry out this manoeuvre.

If possible raise the bed to best working height. Ensure if fitted, bed brakes are applied.

Inform all parties involved on manoeuvre. Confirm person's understanding.

Carer places knee, lower leg and foot on the bed, with their knee at waist/hip height. As close to the person's leg as possible, carer keeping this leg at a right angle.

Carers other foot should be flat on the floor.

Place hand nearest the person over top of their nearest shoulder, carers other hand under the persons shoulder, on shoulder blade. Hands should now be in a 'T' shape.

Carer raises head, looking directly ahead.

Give command Ready, Steady, move. On 'move' carer sits back on their heel raising person into a sitting position.

When sat up carer placing arm nearest person diagonally across their back with hand on bed can maintain person's position.

Alternatively a handling sling can be used **with two carers**

Roll person from side to side position a handling sling under persons shoulders, with loops to the outside.

Carer places knee, lower leg and foot on the bed, with their knee at waist/hip height. As close to the person's leg as possible.

Carers other foot should be flat on the floor.

Carers grasp sling loop with hand nearest person, position other hand flat on bed.

Give command Ready, Steady, move. On 'move' carer sits back on their heel. Raising person into sitting position.

Lying to sitting over side of bed, with slide sheet

If possible raise the bed to best working height.

Ensure if fitted, bed brakes are applied.

Inform all parties involved on manoeuvre. Confirm person's understanding.

Roll person and place slide sheet under the persons buttocks only, depending upon which side of the bed the person will be assisted out from.

Carer places knee, lower leg and foot on the bed, with their knee at waist/hip height. As close to the person's leg as possible. Carers other foot should be flat on the floor.

Raise person into sitting position using, lying to sitting manoeuvre, from previous page. Once in a sitting position, carer One supports the person. Carer Two holds the persons legs and rotates legs in stages over side of bed. Carer One then removes the slide sheet.

From sitting to lying on bed reverse procedure.

With Handling Sling and Slide Sheet

Position a handling sling under persons shoulders, with loops to the outside, by rolling the person from side to side.

Carers grasp sling loop with hand nearest person, position other hand flat on bed

Give command Ready, Steady, move. On 'move' carer sits back on their heel.
Raising person into sitting position

Standing at side of bed other carer places handling sling under person's legs.
Carer grasps both end loops of sling then rotates person's legs over side of bed.

Moving a lying person up/down bed. With two carers

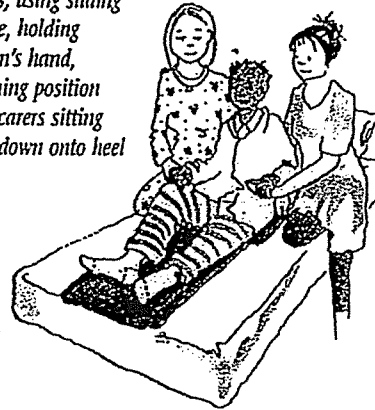
If possible raise the bed to best working height. Ensure if fitted, bed brakes are applied. Inform all parties involved on manoeuvre. Confirm person's understanding. Position a slide sheet under the person, by use of rolling manoeuvre. Carer each side of bed facing opposite direction, in which person is to be moved, places knee of nearest leg on the bed, keeping leg at a 90 degree angle. Other foot flat on floor. Grasp top layer of slide sheet with both hands.

Give command Ready, Steady, slide. Carers sit back on heel sliding client approximately 5-6 inches, ensuring knuckles remain in contact with bed to prevent lifting. Rest and repeat if necessary.

Move up in bed - two carers, using sliding device, holding Person's hand, start position with carers rising up off heel



Move up in bed - two carers, using sliding device, holding Person's hand, finishing position with carers sitting back down onto heel



Moving client up/down bed with slide sheet in sitting position. With two carers

If possible raise the bed to best working height.

Ensure if fitted, bed brakes are applied.

Inform all parties involved on manoeuvre. Confirm person's understanding.

Position a slide sheet under the person, by use of rolling manoeuvre.

Sit person up in bed using the lying to sitting manoeuvre.

Carers facing bottom of bed place lower leg and foot of nearest leg on the bed. Positioning themselves just behind and to one side of person's hip. Other foot firmly on the floor.

Carer's place shoulders behind the shoulders of the person.

Carers using outside hand hold person's hand in 'palm to thumb' / 'palm to palm' or 'elbow' position.

Using hand nearest person grasp hold of top layer of sliding sheet pressing knuckles firmly onto the bed.

Give command Ready, Steady, slide. Carers sit back onto heels pulling back on slide sheet approximately 5 - 6 inches ensuring knuckles remain in contact with bed. Rest and repeat if necessary.

To move person down the bed, on command 'slide' push slide sheet.